

AI-Powered HMS Synthetic Clinical Dataset

Clinical Note

Patient Name	Phan Van Quy	MRN	MRN-0033
DOB	1982-12-11	Gender	Female
Encounter Date	2024-07-15	Department	Cardiology
Attending Provider	Dr. Dev Doctor	Status	active

Synthetic Data Warning: This document is generated for software testing, OCR parsing, chunking, embedding, access-control validation, and Graph RAG demonstrations. It is not valid medical advice and must not be used for patient care.

Subjective

Phan Van Quy is a 43-year-old patient in Cardiology presenting for follow-up of dyspnea on exertion and lower-extremity edema. PMH includes AFib, HTN, dyslipidemia, possible CKD stage 3. Patient reports symptoms are stable overall. Denies severe chest pain, syncope, active bleeding, or new focal neurologic deficit unless otherwise documented. Current meds include apixaban 5mg BID; metoprolol succinate 50mg daily; lisinopril 10mg daily; furosemide 20mg daily. Allergies: penicillin allergy - rash.

Objective

Vitals: BP 143/73 mmHg, HR 95 bpm, RR 15/min, Temp 36.9 C, SpO2 97% RA, weight 88 kg. Exam: alert, oriented, no acute distress. Cardiopulmonary exam without unstable findings. Abdomen soft, non-tender. Extremities show no acute deformity; edema varies by department-specific condition. Labs reviewed below; medication list reconciled.

Recent Lab Snapshot

Date	Analyte	Value	Unit	Reference Range	Status
2026-05-03	Creatinine	1.74	mg/dL	0.7-1.3	High
2026-05-03	eGFR	51.0	mL/min/1.73m2	>=60	Low
2026-05-03	Glucose	153.0	mg/dL	70-99	High
2026-05-03	HbA1c	7.5	%	<5.7	High
2026-05-03	BNP	343.0	pg/mL	<100	High
2026-05-03	AST	27.0	U/L	10-40	Normal
2026-05-03	ALT	31.0	U/L	7-56	Normal
2026-05-03	Potassium	4.5	mmol/L	3.5-5.0	Normal
2026-05-03	Hemoglobin	13.9	g/dL	12.0-17.5	Normal

Assessment

Primary assessment: acute on chronic heart failure with preserved ejection fraction, atrial fibrillation, hypertension. Patient is clinically stable for ongoing management. Risk considerations include renal dosing, medication safety, fall risk, and need for timely escalation if red-flag symptoms occur.

Plan

Continue telemetry if inpatient. Monitor BNP, Cr, eGFR, K, and daily weight. Adjust diuretic dose based on BP and renal function. Provide patient education, update care team via SBAR, and document any access-relevant clinical justification before AI-assisted retrieval of PHI.

Detailed Care Coordination Notes

Problem List

1. acute on chronic heart failure with preserved ejection fraction, atrial fibrillation, hypertension. 2. Medication safety monitoring. 3. Functional status and discharge readiness. 4. Follow-up coordination with Cardiology.

Safety Triggers

Escalate to responsible clinician for SBP < 90, HR > 130, SpO2 < 92%, acute mental status change, active bleeding, severe pain, fever with neutropenia risk, or rapidly rising Cr/K.

AI Retrieval Tags

access_tags: read, summary, labs, medication, cardiology as applicable. Suggested document_type for ingestion: clinical_note.